Paper Name [Citation - APA]	Paper Link	Categories	Topic of Interest - rewrite keywords/frame key implications better	Abstract	Key Points	Relevance
Duckworth, M., Garfield, C. F., Santiago, J. E., Gollan, J., O'Sullivan, K., Williams, D., Lee, Y., Muhammad, L. N., & Miller, E. S. (2024). The design and implementation of a multi-center, pragmatic, individual-level randomized controlled trial to evaluate Baby2Home, an mHealth intervention to support new parents. Contemporary clinical trials, 142, 107571. https://doi.org/10.1016/j.cct.2024.107571	https://pubmed.ncbi.nlm.nih.gov/38740296/	Baby2Home		Becoming a parent is a transformative experience requiring multiple transitions, including the need to navigate several components of health care, manage any mental health issues, and develop and sustain an approach to infant feeding. Baby2Home (B2H) is a digital intervention built on the collaborative care model (CCM) designed to support families during these transitions to parenthood. Objectives: We aim to investigate the effects of B2H on preventive healthcare utilization for the family unit and patient-reported outcomes (PROs) trajectories with a focus on mental health. We also aim to evaluate heterogeneity in treatment effects across social determinants of health including self-reported race and ethnicity and household income. We hypothesize that B2H will lead to optimized healthcare utilization, improved PROs trajectories, and reduced racial, ethnic, and income-based disparities in these outcomes as compared to usual care. Methods: B2H is a multi-center, pragmatic, individual-level randomized controlled trial. We will enroll 640 families who will be randomized to: [1] B2H + usual care, or [2] usual care alone. Preventive healthcare utilization is self-reported and confirmed from medical records and includes attendance at the postpartum visit, contraception use, depression screening, vaccine uptake, well-baby visit attendance, and breastfeeding at 6 months. PROs trajectories will be analyzed after collection at 1 month, 2 months, 4 months, 6 months and 12 months. PROs include assessments of stress, depression, anxiety, self-efficacy and relationship health. Implications: If B2H proves effective, it would provide a scalable digital intervention to improve care for families throughout the transition to new parenthood.	This paper talk about the design and implementation of a randomized control trial for the Baby2Home application. it explains how Baby2Home was translated from a digital-health concept into a real-world clinical trial involving new parents across multiple clinical sites. The team conducted a multicenter, pragmatic, individual-level randomized controlled trial for new parents. The paper presents Baby2Home as an intervention intended to support the multifaceted needs of new parents. The importance of the intervention is that postpartum needs do not belong to a single specialty. Parents may need simultaneous support from obstetrics, primary care, pediatrics, behavioral health, lactation services, social services, and family or community supports.The paper is especially useful as the foundation for determining what data the AI system would eventually consume and what outcomes it should improve.	
Rego, C., Santiago, J. E., Miller, E. S., Garfield, C. F., Gollan, J., O'Sullivan, K., Williams, D., Montague, E., & Lee, Y. S. (2025). Understanding the Implications of mHealth Technology in Collaborative Care Programs and Its Role in Supporting Postpartum Care: Qualitative Interview Study of the Baby2Home Intervention Using the Parallel Journeys Framework. JMIR pediatrics and parenting, 8, e70936. https://doi.org/10.2196/70936	https://pubmed.ncbi.nlm.nih.gov/40857707/	Baby2Home		The postpartum period represents a critical period for both birthing and nonbirthing parents due to mental health concerns and new caregiving demands. Collaborative care models aim to address these needs, but postpartum care remains fragmented, lacking continuity and holistic support. Baby2Home (B2H) is a digital intervention rooted in the collaborative care model, specifically designed to support parents through their transition into parenthood by addressing their physical, emotional, and psychosocial needs. This intervention seeks to close the gaps left by traditional care models by providing continuous, organized, and accessible support throughout the postpartum period. In our qualitative study of the B2H intervention, we reference the parallel journeys framework and use it as a part of our analysis to evaluate whether mobile health (mHealth) technology addresses the holistic needs (postpartum and psychosocial) of new parents.	The paper is an interview study that focuses on applying a parallel journeys framework on new parents to understand the relationships between parents and care managers. Its conceptual contribution is the use of the Parallel Journeys Framework, which recognizes that parents and healthcare professionals move through different but interconnected experiences of postpartum care.This study focuses on the lived experience of using and implementing the technology. The “parallel journeys” concept is useful because the parent’s journey and the care team’s journey are not identical. An effective platform must connect these journeys without treating the parent as merely a data source or the clinician as merely a recipient of alerts. Interface reported by the parents 1. Parent-reported symptoms, questions, concerns, and preferences should not be buried in a message stream. They should be represented as structured, reviewable information. 2. A parent may report a problem without requiring urgent action, whereas another report may require immediate escalation. 3. The system should allow care managers to send information, receive responses, document actions, and close the loop.	
AlSerkal Y, Ibrahim N, Alsereidi A, Ibrahim M, Kurakula S, Naqvi S, Khan Y, Oottumadathil N Real-Time Analytics and AI for Managing No-Show Appointments in Primary Health Care in the United Arab Emirates: Before-and-After Study JMIR Form Res 2025;9:e64936 URL: https://formative.jmir.org/2025/1/e64936 DOI: 10.2196/64936	https://formative.jmir.org/2025/1/e64936	Healthcare Dashboard Design			This paper talks about a way of how AI has been embedded into a system for a dashboard. The AI integrated was suppposed to help reduce the number of no shows for the application using a before-and-after framework. This study evaluated an AI-driven, EHR-linked dashboard designed to predict appointment no-shows and support real-time operational management in a large primary-care network. The article reports an accuracy of approximately 86%. The dashboard combined predictive and operational information, including Predicted no-show risk, Patient appointment information, Patient flow, Waiting times, Clinic volume and many more such factors. The paper therefore demonstrates a closed-loop workflow: Prediction→Worklist→Outreach or operational action→Outcome m onitoring The paper creates a definite principle for other developers to consider - "AI output must be embedded in a visible workflow with a designated user, a defined action, and measurable follow-up."	
Reddy S, Rogers W, Makinen V, Coiera E, Brown P, Wenzel M, et al. Evaluation framework to guide implementation of AI systems into healthcare settings. BMJ Health & Care Informatics. 2021;28:e100444. https://doi.org/10.1136/bmjhci-2021-100444	https://informatics.bmj.com/content/28/1/e100444	AI in Healthcare			This paper proposes a framework for evaluating AI systems as they move from technical development into clinical and operational healthcare settings. Its key contribution is the argument that AI evaluation must extend beyond technical performance. The framework organizes evaluation into three broad domains: 1. Technical capability 2. Clinical or operational utility 3. Adoption and implementation The framework supports evaluating whether the AI actually improves care or workflow. The authors define the success of the AI as - "Better care-manager decisions and more reliable care coordination with acceptable workload and safety, not simply higher model accuracy."	This paper strongly supports a staged research program: Run the model silently and evaluate performance without exposing outputs to care managers. Expose the model to care managers in simulated or limited live workflows. Measure comprehension, perceived burden, and actionability. Evaluate whether the dashboard changes care-manager behavior and improves follow-up. Evaluate parent-level outcomes, equity, safety, resource use, and sustained adoption.

Tun H, Rahman H, Naing L, Malik O Trust in Artificial Intelligence–Based Clinical Decision Support Systems Among Health Care Workers: Systematic Review J Med Internet Res 2025;27:e69678 URL: https://www.jmir.org/2025/1/e69678 DOI: 10.2196/69678	https://www.jmir.org/2025/1/e69678	AI in Healthcare			This paper is a systematic review of the different factors influencing trust of healthcare workers on AI -based clinical descision support systems. 27 studies were analysed and most included research involved physicians and nurses in hospital settings; fewer studies examined primary care, social work, behavioral health, or care management. The review identifies transparency, training, usability, clinical reliability, validation, ethics, human-centered design, customization, and control as central to trust. The paper supports treating explainability and user control as core safety features, not optional interface enhancements.	
Miller, E. S., Jensen, R., Hoffman, M. C., Osborne, L. M., McEvoy, K., Grote, N., & Moses-Kolko, E. L. (2020). Implementation of perinatal collaborative care: a health services approach to perinatal depression care. Primary health care research & development, 21, e30. https://doi.org/10.1017/S1463423620000110	https://pmc.ncbi.nlm.nih.gov/articles/PMC7503171/	Collaborative Care Registry	collaborative care; integrated care; perinatal mental health; perinatal psychiatry	Aim: Our objective was to integrate lessons learned from perinatal collaborative care programs across the United States, recognizing the diversity of practice settings and patient populations, to provide guidance on successful implementation. Background: Collaborative care is a health services delivery system that integrates behavioral health care into primary care. While efficacious, effectiveness requires rigorous attention to implementation to ensure adherence to the core evidence base. Methods: Implementation strategies are divided into three pragmatic stages: preparation, program launch, and program growth and sustainment; however, these steps are non-linear and dynamic. Findings: The discussion that follows is not meant to be prescriptive; rather, all implementation tasks should be thoughtfully tailored to the unique needs and setting of the obstetric community and patient population. In particular, we are aware that implementation on the level described here assumes commitment of both effort and money on the part of clinicians, administrators, and the health system, and that such financial resources are not always available. We conclude with synthesis of a survey of existing collaborative care programs to identify implementation practices of existing programs.	This paper describes how to implement perinatal Collaborative Care for depression in real-world health systems. The authors emphasize that implementation is not strictly linear. Programs must adapt to local staffing, patient volume, referral pathways, clinical culture, available technology, and organizational resources. The paper organizes implementation into three broad stages: 1. Preparation 2. Program launch= The paper describes a workflow in which obstetric patients are screened during pregnancy and postpartum. 3. Program growth and sustainment= The paper emphasizes that the purpose of screening is not simply to identify depression. The system must use repeated measures to determine whether treatment is working The paper supports the use of electronic symptom monitoring through secure surveys, email, or telephone-based follow-up, with frequency determined by symptom severity.	
LaRocco-Cockburn, A., Reed, S. D., Melville, J., Croicu, C., Russo, J. E., Inspektor, M., Edmondson, E., & Katon, W. (2013). Improving depression treatment for women: integrating a collaborative care depression intervention into OB-GYN care. Contemporary clinical trials, 36(2), 362–370. https://doi.org/10.1016/j.cct.2013.08.001	https://pubmed.ncbi.nlm.nih.gov/23939510/	Collaborative Care Model (CoCM)	Setting Adaptation & Clinical Feasibility	Background: Women face elevated rates of depression during key reproductive transitions such as pregnancy, the postpartum window, and menopause. Collaborative care models adapted for mood disorders within specialized obstetrics and gynecology practices had not previously been evaluated. Methods: This randomized controlled trial compared a collaborative care depression intervention to usual care among adult women presenting to OB-GYN clinics with clinically significant depressive symptoms (PHQ-9 $\ge$ 10) and a confirmed diagnosis of major depression or dysthymia. Follow-up extended over 18 months, with changes on the SCL-20 depression scale across 12 months serving as the primary endpoint. Results: The study randomized 205 participants (mean age: 39 years; 57% White, 20% Black/African American, 9% Asian/Pacific Islander, 7% Hispanic, 6% Native American). A substantial majority (99%) met criteria for major depression, while approximately one-third met criteria for dysthymia. Pregnant (4.6%) and postpartum (7.5%) women were represented alongside other reproductive life stages. Conclusions: Integrating a dedicated OB-GYN collaborative care team, uniting a care manager social worker, consulting psychiatrist, and primary OB-GYN physician around regular case reviews and registry tracking, proved clinically viable. Further study of tailored quality-improvement models for women's health settings is necessary.	This paper describes the design, implementation, and evaluation of an adapted collaborative care model for depression integrated directly into obstetrical and gynecologic (OB-GYN) care settings. The authors emphasize that establishing collaborative care in this environment calls for tailored clinical processes to fit the unique care patterns of women across reproductive life stages. The paper structures its implementation and operational workflow into core phases: 1. Model Adjustment and Clinical Setup 2. Patient Identification and Program Enrollment = The workflow establishes systematic screening for women over age 18 applying standardized tools, such as the PHQ-9, identifying those with major depressive disorder or dysthymia throughout critical reproductive phases, including pregnancy, the postpartum period, and perimenopause. 3. Measurement-Based Stepped Care and Ongoing Management = The clinical model does not rely on screening alone; it utilizes ongoing longitudinal monitoring to track treatment response over time, assessed via the SCL-20 scale through 12 to 18 months. The paper supports multidisciplinary care delivery centered around an OB-GYN collaborative care team, comprising a care manager, social worker, a consulting psychiatrist, and an OB-GYN physician, who conduct weekly case reviews and utilize an electronic tracking system to monitor symptom changes and modify treatment protocols.	This article provides foundational evidence for integrating collaborative care into routine obstetrical and gynecological settings, directly informing the clinical architecture that the Baby2Home project builds upon. By demonstrating how a multidisciplinary team, combining an OB-GYN physician, a depression care manager, and a consulting psychiatrist, can use weekly case reviews and registry-based tracking to manage perinatal mood disorders, this study validates the clinical validity of delivering stepped psychiatric care directly within women's health clinics. Baby2Home translates these traditional, clinic-bound collaborative care elements into a contemporary digital format, taking the care manager and tracking systems validated in this paper and extending them through mobile health technology.

Miller, E. S., Jensen, R., Hoffman, M. C., Osborne, L. M., McEvoy, K., Grote, N., & Moses-Kolko, E. L. (2020). Implementation of perinatal collaborative care: a health services approach to perinatal depression care. Primary Health Care Research & Development, 21, e30. doi:10.1017/S1463423620000110	https://www.cambridge.org/core/journals/primary-health-care-research-and-development/article/implementation-of-perinatal-collaborative-care-a-health-services-approach-to-perinatal-depression-care/31A049BD038BE5AB722B6EAD75374241	Collaborative Care Model (CoCM)	Health Services Infrastructure & Program Sustainment	Aim & Background: Perinatal Collaborative Care (P-CoCM) integrates behavioral health services into maternity settings to treat perinatal depression effectively. However, moving from clinical trials to broad real-world adoption calls for rigorous attention to implementation science and health services delivery to preserve core evidence-based principles. Methods: Drawing on practical knowledge through different U.S. perinatal collaborative care initiatives, the authors synthesize implementation strategies across three dynamic, iterative phases: preparation, program launch, and program growth/sustainment. Key Insights: Implementation success depends on customizing workflows to local obstetric clinical culture, patient volume, and available resources rather than following rigid, linear formulas. Core structural requirements include multidisciplinary staffing, specialized psychiatric consultation, universal antenatal and postpartum screening algorithms, and registry-driven measurement-based stepped care. Conclusions: Health systems must commit administrative and financial resources to sustain perinatal collaborative care. Longitudinal outcome tracking and continuous quality improvement are vital to ensure that identified patients actually achieve remission rather than falling through clinical gaps.	This paper describes the real-world operationalization and health services framework for implementing perinatal Collaborative Care models (P-CoCM) to manage perinatal depression in routine clinical settings. The authors emphasize that scaling collaborative care across maternal health systems requires bridging evidence-based clinical practices with practical health services delivery, accounting for clinic workflow constraints, multidisciplinary staffing structures, and patient access barriers. The paper outlines the implementation pathway across key organizational stages: 1. System Readiness and Clinical Integration 2. Identification and Screening Workflows = The framework details clinical pathways for systematic perinatal depression screening across antenatal and postpartum visits, formulating clear referral thresholds and care-manager warm handoffs directly within routine obstetrical encounters. 3. Registry Tracking, Stepped Care, and Program Sustainment = The model stresses that identification alone is insufficient; health systems must implement population-based registries and measurement-based care to continuously track clinical outcomes, escalate or modify treatment for non-responders, and monitor long-term program quality. The paper highlights the critical role of specialized psychiatric consultation, embedded behavioral health care managers, and dedicated care-coordination infrastructure in ensuring continuous, equitable perinatal psychological health support within primary and maternity care systems.	This paper discusses the health-services and structural blueprint for establishing perinatal collaborative care programs, detailing the systemic pathways, clinical preparedness, and screening-to-referral mechanisms that Baby2Home operationalizes in real-life settings. It points out that successful perinatal collaborative care entails overcoming clinic-level bottlenecks, navigating workflow constraints, and moving beyond initial diagnosis into continuous, population-based measurement. Baby2Home directly relates to this system by serving as the practical, scalable vehicle that executes these recommendations, bridging obstetrical identification with longitudinal patient tracking, care navigation, and structured behavioral health support across clinical health systems.
Miller, E. S., Mohr, D. C., Williams, D., Shikany, M., Walsh, T., Winquist, N. W., Mir, Z., Gray, E. L., Smith, S. R., Krause, C., Baez, L. M., & Reddy, M. C. (2026). A technology-enabled service for perinatal collaborative care models: A randomized clinical trial. Pregnancy, 2(3), e70298-n/a. https://doi.org/10.1002/pmf2.70298	https://pmc.ncbi.nlm.nih.gov/articles/PMC13344243/	Collaborative Care Model (CoCM)	Technology-Enabled Care Delivery & Workforce Scalability	Introduction: Perinatal collaborative care improves access to evidence-based mental health services, yet high patient volume, communication barriers, and heavy care-manager workload limit programmatic scalability. This trial evaluated whether integrating a technology-enabled service (TES) improves depression and anxiety symptoms, care engagement, and satisfaction compared to collaborative care alone. Methods: In a randomized controlled trial across five obstetric clinics, 75 pregnant or postpartum individuals with mild-to-moderate depression (PHQ-9 $\ge$ 5) or anxiety (GAD-7 $\ge$ 5) were allocated to either standard perinatal collaborative care or a technology-enabled intervention featuring a cognitive behavioral therapy mobile app, coaching, and a care-manager dashboard. Symptoms were tracked biweekly over 12 weeks. Results: Both study arms showed significant reductions in depression and anxiety over time, with no significant differences between groups in score trajectories, response rates, or remission rates. Stepped-care escalation occurred at virtually identical frequencies (~20% to 21%). However, participant involvement at the midpoint was significantly higher in the technology-enabled arm, with comparable satisfaction levels across groups. Conclusions: While digital enhancements did not produce superior clinical symptom reductions relative to standard collaborative care, they markedly bolstered patient participation and operational workflow. Technology-enabled services offer practical value by supporting workforce scalability and lasting sustainability in perinatal collaborative care systems.	This paper describes how to optimize and scale perinatal Collaborative Care models (P-CoCM) through incorporating a technology-enabled service (TES) directly into routine obstetric workflows. The authors point out that while collaborative care effectively treats perinatal mental health conditions, clinical implementation encounters severe scalability hurdles stemming from care manager burden, communication friction, and the high volume of perinatal patients requiring close longitudinal monitoring. The paper organizes implementation into three broad stages: 1. Preparation and Workflow Optimization 2. Program Launch and Patient Onboarding = The paper describes an integrated clinical workflow within obstetric practices where pregnant and postpartum patients, up to 12 months postpartum, are identified through routine screening, using the PHQ-9 and GAD-7, or clinician/self-referral, followed by collaborative care planning with a licensed care manager. 3. Registry-Driven Stepped Care and Continuous Assessment = The paper emphasizes that clinical improvement relies on measurement-based stepped care rather than initial diagnosis alone. The system tracks patients through a population registry, reassessing symptoms bi-weekly while symptomatic and monthly upon remission to guide specialist-informed treatment modifications until remission is achieved. The paper supports the use of digital tools—specifically a patient-facing mobile application paired with a care manager–facing dashboard—to simplify bi-directional communication, deliver psychoeducational resources, and automate regular symptom assessment without overburdening clinical staff.	This paper is the direct conceptual and technical precursor to the modern Baby2Home framework, as it identifies how technology-enabled services can solve the critical scalability and workflow bottlenecks of perinatal collaborative care. Through tackling care manager burden, communication friction, and the demand for frequent longitudinal monitoring, it justifies the pairing of a patient-facing app with a care-manager registry. Baby2Home directly embodies this technology-enabled collaborative care model by automating remote symptom tracking, delivering educational content, and streamlining bi-directional communication between new parents and their care team to facilitate continuous stepped care without exhausting clinical capacity.
Yang, H., & Lin, D.-M. (2026). Conversational AI for perinatal mental health: promise, limits, and a human-AI stepped-care framework. Frontiers in Psychiatry, 17, 1847854-. https://doi.org/10.3389/fpsyt.2026.1847854	https://pmc.ncbi.nlm.nih.gov/articles/PMC13212461/	AI in Healthcare				

Zimmermann, M., Schaefer, A., Tong, X. J., Palmer, S., Millen, L., Arowosegbe, A., Julce, C., Moore Simas, T. A., Sheldrick, R. C., Mackie, T. I., & Byatt, N. (2025). Perinatal care professional perspectives of screening for perinatal mood and anxiety disorders: a qualitative analysis. Journal of Reproductive and Infant Psychology, 1–24. https://doi.org/10.1080/02646838.2025.2587670	https://www.tandfonline.com/doi/full/10.1080/02646838.2025.2587670	Care Manager Workflow	Perinatal mood and anxiety disorders; screening; questionnaires; health equity; perinatal care	Aims: The goal of this study was to understand perinatal care professionals' perceptions of perinatal mental health screening and determinants of equitable screening implementation. Methods: Study participants were perinatal care professionals (n = 24), including obstetricians/gynaecologists, midwives, doulas, and other healthcare providers. Eligibility criteria required participants to either provide care to patients in the United States insured through Medicaid or other public insurance programs or work in practices serving individuals facing socioeconomic challenges. We conducted semi-structured interviews to explore participants' experiences with screening for perinatal mental health. Thematic analysis was employed to identify emergent themes related to the determinants of screening implementation, with the Health Equity Implementation Framework (HEIF) serving as the organising structure. Results: Perinatal care professionals perceived screening results to facilitate discussion, awareness of mental health, and clinical decision-making. They identified several determinants influencing equitable implementation through impacting perceived screening validity, completion, patient experience, follow-up, and clinical workflows. Determinants aligned with the HEIF and included characteristics of the (1) screening (e.g. modality, frequency, type of tool), (2) perinatal individual, (3) perinatal care professionals, (4) clinical encounter, (5) other individuals involved before or after screening, and (6) context. Conclusion: Perinatal care professionals view screening as an important element of clinical decision-making. Reducing inequities in detecting perinatal mood and anxiety disorders requires offering screenings in patients' preferred languages, improving patientprovider relationships, and training to implement best practices. Future research should examine variations in screening implementation to ensure equitable detection of perinatal mood and anxiety disorders.	This paper discusses the use of screeners and its effects on detecting perinatal mental health disorders from the perspetive of care professionals. It discusses the current gaps in implementation of standardized screeners. For example, screeners are not implemented as a part of doula care due to their lack of understanding of how to process and interpret them. Implications as to how to better implement screeners equitably include: offering screeners in the patient's preferred languages, improving patient-provider relationships, and training to administer the screeners.	Discussion of implementation of screeners and how to increase provision and truthfulness of perinatal persons answering the screeners. Discusses areas of how to expand screener provision via language translation and expansion to other providers. These points reinforce the use of screeners in the Baby2Home app and point to how these could be used in clinical decision making.
Dewantiningrum, J., Andonotopo, W., Lukas, E., Adi Pramono, M., Prabowo, W., Bachnas, M., Hariyasa Sanjaya, I., Putra Wiradnyana, A., Kusuma, A., Gumilar, K., Darmawan, E., Akbar, M., Aldiansyah, D., Suryawan, A., Putra, R., Anwar, A., Nugraha, L., Andanaputra, W., Dharma, W., Sulistyowati, S., Stanojevic, M. & Kurjak, A. (2026). Integrating mental health into obstetric practice: a review of collaborative care models for perinatal anxiety. Journal of Perinatal Medicine, 54(5), 831-847. https://doi.org/10.1515/jpm-2025-0300	https://www.degruyterbrill.com/document/doi/10.1515/jpm-2025-0300/html	Collaborative Care Model (CoCM)	perinatal anxiety; collaborative care; obstetric practice; mental health integration; maternal health policy	Objective: Perinatal anxiety is one of the most common yet least systematically addressed complications of preg- nancy and childbirth. Despite abundant evidence that collaborative and integrated care models improve maternal outcomes, obstetric practice still lacks a defined operational standard for addressing anxiety alongside routine obstetric care. Fragmented screening, insufficient referral systems, and financing barriers continue to delay intervention, widening inequities across populations and settings. Methods: This opinion article synthesizes recent evidence (2010–2025) from PubMed, Google Scholar, and professional guidelines to propose a practical framework for embedding mental health care within obstetric workflows. Drawing upon studies from high- and low-resource contexts, we outline a ten-point minimum standard for perinatal-anxiety management and a three-tier maturity model that describes the progressive integration of collaborative care—from basic screening to digitally supported, team-based systems. The model identifies measurable implementation metrics and policy levers that enable sustainability and equity. Results: Rather than advocating new research, this article translates two decades of findings into a clinically actionable standard. It emphasizes the central role of ob- stetric teams in early detection, stepped care, and follow-up through coordination with mental-health professionals. Conclusions: Integrating mental health into obstetric practice is both a moral and operational imperative. By adopting the proposed minimum standard and maturity model, health systems can transform perinatal anxiety care from discre- tionary innovation to routine expectationachieving faster response, broader access, and better maternal–infant out- comes worldwide.	This paper discusses the integration of anxiety detection into maternal mental health care. Recognizing that perinatal anxiety is a common, biologically consequential condition that remains underdetected and undertreated compared to depression, the authors propose a ten-point minimum operational standard and a three-tier maturity model grounded in the Collaborative Care Model. By combining universal screening with brief psychological therapies, digital tracking tools, care manager coordination, and policy reforms such as bundled payments, the framework seeks to transform fragmented referrals into accountable care. They turn this framework into a three-stepped model to treat anxiety: Tier 1 (Mild Anxiety): Connects patients to universal psychoeducation, community support, and low-intensity digital self-management tools . Tier 2 (Moderate Anxiety): Coordinates or delivers low-intensity behavioral interventions, such as guided self-help, peer support groups, or brief structured cognitive-behavioral strategies . Tier 3 (Severe Anxiety): Facilitates direct connection to specialized psychotherapy and coordinates psychiatric consultation for pharmacotherapy	This relates to how Baby2Home care managers may work to address different levels of anxiety care as this paper is also based on the use of standardized screeners to determine the level of intentsity of anxiety symptoms.
Schmidt, C. N., Alpers, B. S., Patel, D., Chehab, L., Thomas, M., Doshi, N., Briscoe, H., Spaulding, M., Ocegueda, L., & Sammann, A. (2026). Improving care coordination for socially complex perinatal patients: A human-centered design study. Pregnancy, 2(1), e70233-n/a. https://doi.org/10.1002/pmf2.70233	https://obgyn.onlinelibrary.wiley.com/doi/full/10.1002/pmf2.70233	Collaborative Care Model (CoCM)	care coordination, human-centered design, informational continuity, prenatal care, safety-net hospital, transitions of care	Introduction: Designing novel solutions to address care fragmentation in the perinatal ecosystem requires rigorous evaluation of current workflows and engagement of the wide net of stakeholders supporting patient care. Human-centered design (HCD) is an established methodology for identifying challenges faced by patients and providers in clinical workflows, with the goal of producing actionable insights to inform interventions. This study used an HCD approach to (1) understand transitions of perinatal care within a safety-net system and (2) identify barriers that informed design opportunities to improve care coordination for socially complex pregnant patients. Methods: We conducted semi-structured interviews with patients, clinical providers (physicians, midwives, nurses, social workers), and community-based organization (CBO) staff working in a safety-net healthcare system. Interviews were analyzed using an HCD approach to inductive thematic analysis. Results: Thirty-four interviews were conducted with 42 participants, including 13 patients, 22 clinical providers, and seven community-based or public health stakeholders. We developed six insight statements, with themes centered around fragmented communication, limited integration between clinical and community systems, uncertainty surrounding information-sharing policies, and inconsistent implementation of substance-use-related protocols. Participants also highlighted disruptions to continuity of care and variability in discharge planning that contributed to confusion and eroded trust in the care team. Conclusions: Using an HCD approach, we identified key challenges in care coordination for socially complex perinatal patients. Interventions that seek to strengthen informational continuity between clinical and community-based providers offer promise for improved perinatal care delivery in safety-net systems.	In this paper, the researchers identified six key insights highlighting fragmented communication, siloed community resources, data-sharing hesitancy, ambiguous substance use policies that seed bias, rotating provider models that erode trust, and discharge practices that create a false sense of security regarding follow-up appointments. To address these breakdowns, the study generated actionable design opportunity statements focused on closed-loop bidirectional communication, centralized resource tracking, standardized consent workflows, and pre-discharge appointment scheduling. Overall, the paper demonstrates that optimizing informational continuity across existing clinical and community ecosystems offers a sustainable, high-impact strategy to improve perinatal care without requiring redundant new programs	

Cooper, K. M., Masters, G. A., Moore Simas, T. A., & Byatt, N. (2022). The role of clinician assistants in addressing perinatal depression. Journal of Psychosomatic Obstetrics and Gynaecology, 43(3), 368–373. https://doi.org/10.1080/0167482X.2021.1975676	https://www.tandfonline.com/doi/full/10.1080/0167482X.2021.1975676	Care Manager Workflow	Perinatal metal health; health services research; clinician assistants; collaborative care; perinatal depression; postpartum depression	Background: Upwards of one in seven individuals experience perinatal depression and many individuals cannot access treatment. In response, perinatal depression is increasingly being managed in the obstetric setting. This study aimed to characterize the experiences of clinicians and clinician assistants to inform the extent to which clinician assistants can help address depression in obstetric settings. Methods: This cross-sectional analysis used data from an ongoing cluster randomized control trial: The PRogram In Support of Moms (PRISM). Participants included clinicians (physicians, certified nurse midwives, nurse practitioners) and clinician assistants (medical assistants, nursing assistants). Baseline data regarding practices and attitudes of clinicians and clinician assistants toward addressing depression in the obstetric setting were described. Logistic regressions were used to examine the association of clinician time to complete work and depression management. Results: Clinician assistants experienced significantly fewer time constraints than did clinicians. However, having adequate time to complete work was not significantly associated with increased depression management in clinicians. Clinician assistants reported feeling that addressing depression is an important part of their job, despite variation in doing so. Conclusion: Clinician assistants are interacting with perinatal women extensively and are a vital part of obstetric care workflows. Clinician assistants report that they want to address depression and have time to do so. Thus, clinician assistants may be poised to help address the mental health needs of perinatal individuals.		
Özhüner, Y., & Özhüner, E. (2026). The Role of Artificial Intelligence in Improving Maternal and Fetal Health during the Perinatal Period: A Traditional Review. Mediterranean Nursing and Midwifery, 6(2), 82–90. https://doi.org/10.4274/MNM.2025.24318	https://mediterr-nm.org/index.php/pub/article/view/151/181	AI in Healthcare	Artificial intelligence, fetus, health, mother, pregnancy	Objective: This traditional review aims to explore the role of artificial intelligence (AI) in maternal and fetal health during the perinatal period, examining its applications in obstetrics and the implications for healthcare professionals. Method: A comprehensive literature review was conducted to analyze the current applications and potential of AI in improving maternal and fetal health outcomes throughout pregnancy and the postpartum period. The researcher conducted a literature review using appropriate keywords in the PubMed, Google Scholar, ScienceDirect, Web of Science, and Cochrane Library databases from 2011 to 2024. Results: Artificial intelligence technologies show significant potential in enhancing diagnostic accuracy in clinical settings by identifying complex medi-cal data in fetal and maternal health, assisting in risk assessment, and enabling early detection of complications. These applications facilitate early preg-nancy detection, genetic screening, continuous monitoring of maternal health parameters with real-time alerts for deviations, and early detection of fetal abnormalities through advanced imaging. The application of AI during pregnancy, childbirth, and the postpartum period enhances the quality of care and patient satisfaction, while also raising important ethical considerations regarding patient privacy, data security, and decision-making responsibility. Conclusions: The AI applications are transforming clinical practices in maternal and fetal health through advanced diagnostic capabilities, predictive modeling, and personalized care interventions, thereby contributing to the reduction of morbidity and mortality rates for both mothers and babies and the improvement of health outcomes. Keywords: Artificial intelligence, fetus, health, mother, pregnancy	This review synthesizes applications of artificial intelligence across obstetrics, maternal-fetal medicine, and perinatal metabolic health (Mediterranean Journal of Nutrition and Metabolism, 2024). The paper details how predictive algorithms and machine learning models are used to stratify risks for conditions such as preeclampsia, gestational diabetes, and fetal distress via automated cardiotocography analysis. It emphasizes a clinical shift from reactive care to continuous, data-driven profiling by integrating multi-modal inputs, including maternal medical histories, biometric sensor data, and laboratory panels. The review highlights remaining implementation challenges, particularly model generalizability across diverse racial and ethnic populations, data privacy concerns, and seamless integration into primary electronic health record systems.	

Alkhateeb M, Nayeem A, Ahmed A, Alsahli M, Sheikh J, Abd-Alrazaq A AI for Detecting and Predicting Postpartum Depression: Scoping Review J Med Internet Res 2026;28:e77376 URL: https://www.jmir.org/2026/1/e77376 DOI: 10.2196/77376	https://www.jmir.org/2026/1/e77376/	AI in Healthcare	postpartum depression; maternal mental health; perinatal depression; artificial intelligence; machine learning; deep learning; natural language processing; prediction models; computer-aided diagnosis	Abstract Background: Postpartum depression (PPD) affects up to 20% of mothers globally. Early detection is vital for better outcomes, yet screening lacks scalability and predictive power. Artificial intelligence (AI)—through machine learning, deep learning, and natural language processing—enhances the early identification of mothers at risk with greater accuracy. Objective: This study aims to systematically map the existing literature on AI-based methods for detecting and predicting PPD. Methods: This scoping review was conducted in accordance with the PRISMA-ScR (Preferred Reporting Items for Systematic Reviews and Meta-Analyses Extension for Scoping Reviews) guidelines. We included empirical studies that applied AI techniques to detect or predict PPD and were published in peer-reviewed journals, conference proceedings, or dissertations. Studies were excluded if they were nonempirical (eg, reviews, editorials, and abstracts), not published in English, focused on general perinatal mental health without a specific emphasis on PPD, or used AI solely for monitoring or treatment rather than prediction or detection. We systematically searched 8 databases—MEDLINE, Embase, PsycINFO, CINAHL, Scopus, IEEE Xplore, ACM Digital Library, and Google Scholar—from inception through February 28, 2025. The search strategy was supplemented by backward and forward reference screening and biweekly alerts to capture newly published studies. Two independent (M [Alkhateeb] and A [Nayeem])reviewers (M [Alkhateeb] and A [Nayeem]) screened the retrieved studies, with disagreements resolved by a third reviewer (AA [Alrazaq]). Data were extracted by 2 independent reviewers using a standardized extraction form capturing study characteristics, AI model types, data sources, features, preprocessing, validation strategies, and performance metrics. A formal risk-of-bias assessment was not performed due to the scoping nature of the review. All extracted data were synthesized narratively. Results: Out of 503 retrieved studies, 65 met the inclusion criteria. The United States contributed the largest proportion of studies (18/65, 27.7%). The highest number of publications occurred in 2024 (17/65, 26%). Most included studies were journal articles (46/65, 71%). Short-term postpartum outcomes (≤12 weeks) were most frequently assessed (20/65, 30.8%). Most included studies (52/65, 80%) applied AI models for predicting PPD, while 14 of 65 (22%) studies used them for detection. Sociodemographic data were most frequently used (49/65, 75.4%), followed by psychological data (44/65, 68%) and obstetric data (35/65, 55%). Data preprocessing mostly relied on basic scaling (51/65, 79%) and some missing data imputation (29/65, 44.6%). Machine learning dominated (57/65, 87.7%), especially random forest, support vector machines, and logistic regression. Internal validation (k-fold, hold-out) was standard, while external validation was scarce. Ensemble-based boosting models consistently demonstrated superior performance across key metrics, highlighting their potential for accurate and scalable PPD prediction. Current studies suffer from limited sample sizes, geographic bias, lack of standardized feature sets, minimal external validation, and inconsistent reporting of comprehensive model metrics. Conclusions: This scoping review analyzes 65 studies on AI in PPD, highlighting dominant use of classical machine learning, limited deep learning adoption, underuse of advanced preprocessing, inconsistent validation, and reliance on structured, unimodal data—mainly sociodemographic, clinical, and obstetric features.	Mapping 65 empirical studies across eight major databases through early 2025, this scoping review evaluates the current state of artificial intelligence methods used to predict and detect postpartum depression (Journal of Medical Internet Research, 2026). The analysis reveals that 87.7% of published studies rely on traditional machine learning algorithms—notably Random Forest, Support Vector Machines, and Logistic Regression—with ensemble boosting techniques consistently yielding the highest accuracy. Predictive models overwhelmingly rely on structured, unimodal data, primarily sociodemographic (75.4%), psychological (68%), and obstetric (55%) records. The review identifies critical gaps in the literature, including a severe lack of external model validation, geographic sampling bias, brief outcome tracking windows (≤ 12 weeks postpartum), and underutilization of deep learning and natural language processing on unstructured clinical notes.	

V. K. Mishra, B. V. Tadikonda, M. Rajwade and V. Viradia, "Agentic AI for Closing Gaps in Care: A Conceptual Architecture for Automating Gap-in-Care Workflows," SoutheastCon 2026, Huntsville, AL, USA, 2026, pp. 1-6, doi: 10.1109/SoutheastCon63549.2026.11476134. keywords: {Feeds;Antennas;Feedback;Filtering;Circuits;Central Processing Unit;Filters;Internet;Routing;Radio access networks;Gaps in care;value-based payment;HEDIS;clinical quality measures;automation;multi-agent systems;agentic AI;health informatics},	https://ieeexplore.ieee.org/abstract/document/11476134	AI in Healthcare		Gaps in care, defined as missed or delayed evidence-based services such as screenings, laboratory tests, chronic disease follow-up, and medication adherence, remain a major driver of preventable morbidity, mortality, and cost. Surveys suggest that a large majority of adults delay preventive screenings and missed prevention opportunities are estimated to cost the U.S. healthcare system around $ 55 billion annually, which is roughly 30 cents of every healthcare dollar [1], [2]. Patients with multiple chronic conditions account for 30% of Medicaid beneficiaries but 83% spending, often related to complications that might be mitigated by timely care [3]. In parallel, payers operationalize gap closure through quality programs such as HEDIS and Star Ratings and tie increasing proportions of value-based payments to gap-closure performance [4], [5], [22]. Yet, current workflows for managing gaps in care are highly manual and fragmented, relying on batch gap lists, spreadsheet-based reconciliation, manual outreach, and an end-of-year “HEDIS season,” which contributes to an administrative burden and clinician burnout while delivering an inconsistent impact on outcomes and equity. In this research paper, we formulate an industrial, systemlevel problem statement for gap-in-care management and propose conceptual, agentic artificial intelligence (AI) architecture to automate and orchestrate gap-closure workflows across payers, providers, and partner organizations. We synthesize challenges described in existing gap-in-care, HEDIS, and value-based care literature—data fragmentation, misaligned measure logic and attribution, batch and manual workflows, poor workflow integration, patient engagement barriers, and limited equity feedback—and derive requirements for an end-to-end technical model. We then present a Payment-Aware Gap Orchestration (PAGO) model, featuring agents for data ingestion and identity resolution, contract and measure parsing, real-time gap computation and prioritiz...		
Reddy S, Rogers W, Makinen V, Coiera E, Brown P, Wenzel M, et al. Evaluation framework to guide implementation of AI systems into healthcare settings. BMJ Health & Care Informatics. 2021;28:e100444. https://doi.org/10.1136/bmjhci-2021-100444	https://papers.ssrn.com/sol3/papers.cfm?abstract_id=6871438	AI in Healthcare		Maternal and child health is the most demanding test of any health system planning framework, and the highest priority of most public health systems. This paper demonstrates that the four-quadrant operational framework - which classifies healthcare by acuity (Acute vs Non-Acute) and intervention dependency (Interventional vs Non-Interventional) rather than by institutional tier or medical specialty - maps onto the entire MCH continuum cleanly and exhaustively, and in doing so localizes precisely where most maternal and neonatal mortality gaps lie and how it should be closed. Three findings emerge. First, the majority of MCH services are Non-Acute Non-Interventional (Q3) - antenatal care, immunization, nutrition, counselling, postnatal follow-up - and are therefore distributable through digital and community channels. This provides a theoretical foundation for the ASHA and ANM workforce that the existing tier system never supplied. Second, the mortality gap lies overwhelmingly in the Acute Interventional quadrant (Q2): mothers and newborns die not from a failure of knowledge or coverage but from distance to emergency obstetric and neonatal care at the moment of crisis. Third, the correct response is not to build dedicated, single-purpose maternal facilities but to ensure that every population cluster has access, within a defined travel time, to a non-sectoral acute-care facility capable of handling all emergencies - obstetric, neonatal, cardiac, and trauma - connected to community and home settings by reliable emergency transport. Fragmented, single-purpose facilities are shown to be an artifact of fragmented financing rather than a clinical necessity, and to work against the volume thresholds on which good acute outcomes depend. The paper confronts the two hardest objections to this position - the tight time-to-death window of obstetric hemorrhage, and the principle that childbirth is a physiological event rather than a disease - and shows that both are answered within the framework without new categories. Throughout, it treats implementation as a matter of mapping and reorganizing existing infrastructure rather than building anew, and it closes with a generic methodology for applying and testing the framework in any region.	To bridge the gap between technical AI development and real-world clinical adoption, an international panel of experts developed the Translational Evaluation of Healthcare AI (TEHAI) framework (BMJ Health & Care Informatics, 2021). Built on health technology assessment principles and translational research models, TEHAI evaluates health AI across three core dimensions comprising 15 subcomponents: Capability (dataset integrity, validation, and performance metrics), Utility (usability, efficiency, safety, and ethical factors like explainability and privacy), and Adoption (workflow integration, governance, and generalizability). Unlike traditional evaluation instruments that focus strictly on offline model accuracy, TEHAI provides a practical, standardized blueprint for evaluating AI solutions at any stage of development or clinical deployment.	

Turco, A., Chu, A., Feinberg, E., Ward, L. G., Elwy, A. R., & Miller, E. S. (2026). Scaling the collaborative care model in obstetrics: Multilevel determinants to guide implementation. Pregnancy, 2(4), e70356. https://doi.org/10.1002/pmf2.70356	https://pmc.ncbi.nlm.nih.gov/articles/PMC13344441/	Care Manager Workflow	Obstetric workflow barriers, clinical buy-in, and handoff pathways	Background: Perinatal mood and anxiety disorders affect a substantial portion of pregnant and postpartum individuals, yet barriers in traditional obstetric settings limit access to mental health care. The Collaborative Care Model (CoCM) integrates behavioral health into routine medical care, but scaling it within obstetrics poses distinct implementation challenges. Methods: Using implementation science frameworks, such as the Consolidated Framework for Implementation Research [CFIR], this study evaluated multi-level determinants influencing the embedding of CoCM across outpatient obstetrics and gynecology clinics through qualitative interviews and stakeholder observations. Results: Implementation determinants spanned outer-setting factors, including stigma around psychotropic medications during pregnancy/lactation and Medicaid coverage constraints; inner-setting factors, including limited physical clinic space, fast-paced clinic flow, and competing clinical demands; and individual provider attitudes, including obstetric clinician comfort with mental health prescribing. Key workflow friction centered on warm handoffs and the scope of the care manager's role. Conclusions: Successfully scaling CoCM in obstetrics requires explicit implementation strategies, including tailored clinical processes, dedicated care-manager coordination, stakeholder support, and cross-disciplinary education between obstetric clinicians and behavioral health specialists.	Multi-Level Barriers in OB-GYN Settings: Highlights clinical integration barriers, including limited physical clinic space for behavioral health staff, competing obstetrical priorities, and perinatal provider discomfort with psychotropic medication management. Warm Handoff Bottlenecks: Emphasizes that fragmented clinic-to-care-manager handoffs and rigid clinic hours cause high patient drop-off between routine prenatal visits and mental health intake. Clinical Culture & Buy-In: Identifies that cross-training obstetric clinicians and normalizing routine behavioral health screening are critical to establishing the care manager as an integral clinic team member. Connection to Baby2Home: Baby2Home targets these clinic-level bottlenecks by providing an asynchronous digital platform that connects parents to care managers outside the clinic, sustaining continuous engagement even when in-person handoffs fail.	This study details how clinical integration barriers, limited clinic space, and rushed clinic visits frequently disrupt the warm handoffs necessary to engage obstetric patients in collaborative care. Baby2Home directly targets these clinic-level barriers by delivering an accessible mobile platform that connects patients and care managers outside the physical office. By sustaining continuous communication beyond routine appointments, Baby2Home prevents vulnerable postpartum patients from falling through the cracks when in-person clinic handoffs fail.
MITTAL, A., JONES, T., KARKAR, R., SUH, J., WILLIAMS, S., ZHENG, Y., ANDRIS, L. M., BATES, N., BAUER, A. M., LOSTUTTER, T. W., FANN, J. R., FOGARTY, J., & HSIEH, G. (2025). SCOPE: Examining Technology-Enhanced Collaborative Care Management of Depression in the Cancer Setting. Proceedings of the ACM on Human-Computer Interaction, 9(2), CSCW162. https://doi.org/10.1145/3711060	https://pmc.ncbi.nlm.nih.gov/articles/PMC12165027/	Care Manager Workflow	Digital dashboards, mobile apps, and remote symptom monitoring	Background: In complex medical environments with high symptom burdens, behavioral health care managers handle considerable workloads in tracking symptoms and coordinating stepped care. Technology-enhanced collaborative care provides the potential to simplify patient-provider communication and optimize care management. Methods: The Symptom Care Online Patient Engagement (SCOPE) project evaluated an integrated digital platform comprising a patient-facing mobile tool, for mood logging, symptom reporting, and behavioral activation, coupled with a clinician-facing electronic registry dashboard embedded in an oncology collaborative care program. Results: Care managers reported that real-time, patient-generated health data streamlined clinical preparation, facilitated targeted between-visit interactions, and enabled proactive stepped-care escalation. Patients reported increased empowerment, self-monitoring awareness, and an enhanced sense of continuous connection with their care team between formal encounters. Conclusions: Coupling patient-facing digital tools with a clinical registry lowers administrative and communication friction for care managers, demonstrating that technology-enhanced collaborative care is able to enhance care-manager workflow effectiveness and sustain patient engagement across parallel physical and psychological medical journeys.	Dual-Platform Architecture: Evaluates the relationship between a patient-facing mobile app, delivering behavioral activation exercises and mood logging, and a provider-facing clinical registry dashboard. Streamlined Clinical Processes: Finds that patient-generated health data allows care managers to prepare more efficiently for clinical encounters, reducing recall time during visits and attending to urgent patient needs. Enhanced Between-Session Engagement: Demonstrates that digital tools maintain a sense of continuous connection between patients and providers between scheduled appointments. Connection to Baby2Home: Baby2Home leverages this exact dual-platform architecture, combining a parent-facing mobile application with care-manager dashboards to track recovery progressions and prompt interventions.	This paper demonstrates that pairing a patient-facing mobile application with a clinical registry dashboard streamlines care-manager workflows and maintains patient engagement between visits. Baby2Home relies on this exact dual-platform architecture, enabling new parents to log symptoms and access resources while care managers track longitudinal recovery data. By allowing care managers to monitor recovery progressions remotely, Baby2Home enables prompt clinical escalation without exhausting clinician bandwidth.

LePoire E, Joseph M, Heald A, Gadbois D, Jones A, Russo J, Bowen DJ. Barriers and facilitators to collaborative care implementation within the New York State Collaborative Care Medicaid Program. BMC Health Serv Res. 2024 Apr 24;24(1):505. doi: 10.1186/s12913-024-10909-0. PMID: 38654291; PMCID: PMC11040977.	https://pubmed.ncbi.nlm.nih.gov/38654291/	Care Manager Workflow	Medicaid billing, documentation burden, and reimbursement viability	Background: The New York State Collaborative Care Medicaid Program (NYS CCMP) established financial reimbursement pathways to support CoCM delivery for Medicaid-insured populations. However, large-scale adoption and sustainability vary widely across clinical sites. Methods: A qualitative evaluation of clinic leaders, behavioral health care managers, and administrative personnel participating in the NYS CCMP was conducted to identify system-wide barriers and facilitators associated with model adoption, billing, and long-term sustainment. Results: Major implementation barriers included cumbersome monthly billing documentation rules, strict time-tracking requirements, low or delayed Medicaid reimbursement rates, and significant care-manager turnover. Conversely, critical facilitators included strong leadership support, centralized billing/administrative support teams, integrated electronic health record (EHR) registry tools, and dedicated continuous training for care managers. Conclusions: While state-level reimbursement structures are critical for CoCM viability, successful operation requires mitigating administrative billing burdens on care managers and providing structural clinic infrastructure to ensure program sustainability for underserved populations.	Medicaid Billing Complexity: Identifies that strict monthly time-tracking, complex documentation rules, and differing payer billing structures create substantial administrative overhead for care managers. Reimbursement Disconnect: Highlights that low or unpredictable Medicaid reimbursement rates undermine clinic financial viability, making it difficult to sustain dedicated care-manager staffing. Need for Infrastructure Support: Demonstrates that clinics with dedicated administrative billing staff and strong leadership buy-in achieve higher program retention and fidelity relative to under-resourced practices. Connection to Baby2Home: Baby2Home's pragmatic trial design explicitly evaluates health equity across diverse socioeconomic and Medicaid populations, gathering utilization and engagement data geared toward optimizing care-manager billing and programmatic sustainability.	This evaluation highlights how complex documentation, rigid time-tracking, and uneven Medicaid reimbursement threaten the operational sustainability of collaborative care programs. Baby2Home deals with these real-world implementation challenges by deploying a pragmatic trial across a range of socioeconomic and Medicaid-insured populations. Via systematic collecting utilization and engagement data, Baby2Home provides the empirical foundation needed to establish sustainable reimbursement and scalable collaborative care delivery.
Melville JL, Reed SD, Russo J, Croicu CA, Ludman E, LaRocco-Cockburn A, Katon W. Improving care for depression in obstetrics and gynecology: a randomized controlled trial. Obstet Gynecol. 2014 Jun;123(6):1237-1246. doi: 10.1097/AOG.0000000000000231. PMID: 24807320; PMCID: PMC4052378.	https://pubmed.ncbi.nlm.nih.gov/24807320/	Collaborative Care Model (CoCM)	Clinical trial efficacy and symptom remission in OB-GYN settings	Background: Depression is prevalent in women attending obstetrics and gynecology (OB-GYN) clinics, but standard care frequently fails to achieve adequate treatment response or remission. Methods: In a randomized controlled trial, the DAWN study, 205 adult women with major depression or dysthymia attending university-affiliated OB-GYN clinics were randomized to an adapted collaborative care intervention or usual care. The intervention paired an on-site depression care manager, social worker, an OB-GYN physician, and a consulting psychiatrist who utilized stepped care, Problem-Solving Therapy and/or antidepressant medications, guided by an electronic registry. Outcomes were tracked over 18 months. Results: At 12 months, women in the coordinated care arm demonstrated significantly greater decreases in depressive symptoms on the SCL-20 compared to usual care ($p < .001$). Intervention participants were significantly more likely to achieve treatment response (>= 50% symptom reduction) and complete depression-specific visits, with sustained clinical gains observed through 18 months. Conclusions: Collaborative care adapted for OB-GYN clinics produces superior, maintained improvements in depressive symptoms and patient satisfaction compared to standard care, establishing the clinical effectiveness in integrating multidisciplinary stepped-care behavioral health teams into routine women's healthcare.	Longitudinal Symptom Remission: Demonstrated in an RCT, the DAWN study, that women receiving collaborative care had significantly greater improvements in depressive symptoms at 12 and 18 months compared to usual care. Functional & Quality-of-Care Gains: Participants in the collaborative care arm were 74% more likely to achieve at least a 50% symptom reduction, attended more specialty behavioral health sessions, and reported higher treatment satisfaction. Stepped-Care Actionability: Proved that structured psychiatric case reviews paired with stepped treatment modifications, modifying antidepressant doses or augmenting with Problem-Solving Therapy, work effectively within outpatient OB-GYN clinics. Connection to Baby2Home: Melville et al. verified that including collaborative care teams into women's health settings successfully treats depression; Baby2Home scales this proven model beyond the physical clinic walls directly into the postpartum home.	This randomized trial established the clinical efficacy of collaborative care in women's health settings, displaying significant reductions in depression symptoms when care managers and consulting psychiatrists support OB-GYN clinics. Baby2Home builds directly on this clinical foundation by adapting these proven multidisciplinary team roles into a modern digital format. Rather than confining evidence-based stepped care to physical clinic encounters, Baby2Home extends this validated collaborative support directly into the patient's home.

Ferrario CA, Shannon C, Petrovic N, Gray S, Linonis R, Rao R, Han CS, Krakow D, Richards M. Collaborative Care in Perinatal Mental Health: Implementation and Insights From the UCLA MOMS Clinic. J Psychiatr Pract. 2025 Nov 1;31(6):312-318. doi: 10.1097/PRA.0000000000000891. PMID: 41325174.	https://pubmed.ncbi.nlm.nih.gov/41325174/	Collaborative Care Model (CoCM)	Real-world perinatal translation across five clinical pillars	Background: Perinatal mood and anxiety disorders require specialized, multi-systemic interventions that account for maternal, fetal, and infant well-being. Real-world implementation models are needed to translate trial-based CoCM protocols into routine outpatient maternity care. Methods: This report describes the clinical development, operational structure, and evaluative implementation outcomes of the UCLA Maternal Outpatient Mental-health Services (MOMS) Clinic, an embedded perinatal collaborative care model serving preconception, pregnant, and postpartum patients. Results: The clinic operationalizes perinatal CoCM across five core clinical pillars: primary obstetric care integration, multidisciplinary coordinated support, dedicated attention to the parent-infant relationship/dyadic bonding, trauma-informed clinical care, and measurement-based care applying standardized registries. The workflow accommodates stepped pharmacotherapy and psychotherapy while tackling infant attachment and family functioning. Conclusions: Real-world perinatal collaborative care succeeds by expanding past individual maternal symptom reduction to address the parent-infant dyad and familial context. Structuring clinical processes around these multi-pillar principles maintains equitable, comprehensive, and sustainable perinatal behavioral health integration.	Moving Beyond Clinical Trials: Details the translation of perinatal CoCM into a routine, sustained outpatient OB-GYN practice (UCLA MOMS Clinic) serving preconception, pregnant, and postpartum patients. Five Core Clinical Pillars: Operationalizes delivery around five evidence-based principles: primary care integration, coordinated care support, direct focus on the parent-infant relationship, trauma-informed principles, and measurement-based care. Dyadic & Relational Health: Emphasizes that perinatal mental health cannot be treated in isolation from infant bonding, parenting transitions, and family relationships. Connection to Baby2Home: Baby2Home operationalizes these pillars, especially parent-infant relationship dynamics along with coordinated support, by actively including nonbirthing parents and simultaneously tracking maternal and infant health outcomes.	This implementation paper demonstrates that perinatal collaborative care is most effective when it moves beyond isolated maternal symptom reduction to embrace dyadic attachment and family-centered support. Baby2Home operationalizes these clinical pillars through actively engaging nonbirthing partners and simultaneously tracking both parental mental health and infant wellness milestones. By supporting the intersecting recovery paths of both parents and their infant, Baby2Home translates the five-pillar collaborative care model into daily family life.
Chang, E., Lewkowitz, A. K., Unger, J. A., Garfield, C. F., & Miller, E. S. (2025). Smartphone Applications to Support Perinatal Mental Health. Obstetrics & Gynecology, 147(2), 229–238. https://doi.org/10.1097/AOG.0000000000006139	https://journals.lww.com/greenjournal/fulltext/10.1097/aog.0000000000006139~smartphone-applications-to-support-perinatal-mental-health	Baby2Home	Fragmented perinatal apps and the case for evidence-based integration	**Background**: Perinatal mood and anxiety disorders are common and undertreated, and perinatal individuals increasingly turn to smartphone applications for support. Yet the clinical value of these consumer apps is uncertain, and it is unclear which features are grounded in evidence-based strategies rather than general education or self-help. **Methods**: This narrative review used systematic search methods to identify smartphone applications marketed to perinatal individuals iin the United States on the Apple App Store and Google Play Store that aimed to improve mental health outcomes. Apps were assessed for whether they had been evaluated in clinical trials or observational studies, for the features they described as supporting perinatal mental health, and for whether those features rested on evidence-based strategies, with the goal of synthesizing the characteristics shared by evidence-based apps. **Results**: Although many apps served a supportive or educational role, few incorporated validated clinical functionality. Only about 13% included validated screening tools for perinatal depression or anxiety, and none included robust mechanisms to monitor treatment response or to sustain user engagement, such as dynamic symptom tracking linked to clinical feedback. The overall functionality of the available apps was fragmented, with supportive features rarely integrated with validated screening or clinical follow-up. **Conclusions**: The current perinatal mental-health app landscape leaves a clear gap between supportive content and evidence-based clinical care. Future digital interventions should unify validated screening, treatment-response monitoring, and evidence-based engagement strategies within a single platform to meaningfully and sustainably improve perinatal mental health outcomes.	- Narrative review using systematic search methods of U.S. perinatal mental-health apps on the Apple App Store and Google Play Store. - Assessed three things: whether apps were tested in trials/observational studies, what features they offered, and whether those features were evidence-based. - Only ~13% of apps included validated screening tools for perinatal depression or anxiety. - None included robust mechanisms to monitor treatment response or to sustain engagement (e.g., dynamic symptom tracking linked to clinical feedback). - Most apps play a supportive or educational role, but functionality is fragmented and rarely integrated with clinical follow-up. - Calls for future apps to unify validated screening, treatment-response monitoring, and evidence-based engagement, effectively Baby2Home's design rationale. - Authored by the Baby2Home lineage team (Garfield, Miller); serves as the anchor citation and pre-written gap statement for the Baby2Home category.	

Pinxiang Wang, Hanqi Chen, Zhouyu Li, Wenyao Xu, Yu-Ping Chang, Huining Li, Continuous prediction of user dropout in a mobile mental health intervention program: An exploratory machine learning approach, Smart Health, Volume 36, 2025, 100565, ISSN 2352-6483, https://doi.org/10.1016/j.smhl.2025.100565.	https://www.sciencedirect.com/science/article/pii/S2352648325000261	AI in Healthcare	Continuous dropout prediction from digital behavioral traces	**Background**: Mobile mental health intervention programs often span several months, during which users may lose interest and fail to complete treatment. Because dropout undermines outcomes, accurately predicting disengagement is essential for delivering timely measures that keep users in care — but dropout is dynamic and difficult to anticipate from static, single-point assessments. **Methods**: This study developed a temporal deep-learning approach to predict user dropout on a continuous basis, using data augmentation and feature-engineering techniques. Interaction metrics from user behavior logs were integrated with semantic features drawn from users' self-reflections across a nine-week intervention program, and temporal models — LSTM, BiLSTM, and GRU architectures — were trained to characterize behavioral patterns over time and predict impending dropout. **Results**: The temporal models achieved high predictive performance, with AUC exceeding 0.95 and balanced accuracy above 0.92 under five-fold cross-validation. Combining behavioral-log interaction data with semantic features from self-reflections effectively captured users' evolving engagement patterns, and the continuous formulation identified rising dropout risk as it developed rather than only after disengagement occurred. **Conclusions**: Temporal deep-learning models can predict user dropout in mobile mental health programs continuously and accurately, supporting timely intervention before users disengage. Such predictive capability offers a practical basis for engagement-preserving alerts that could flag at-risk patients for proactive outreach within a care-management workflow.	- Predicts user dropout in a mobile mental health program continuously, not at a single point in time. - Integrates two data sources: interaction metrics from behavior logs + semantic features from users' self-reflections. - Uses temporal deep-learning models (LSTM, BiLSTM, GRU) with data augmentation and feature engineering. - Nine-week intervention window used to model evolving engagement patterns. - Strong predictive performance: AUC > 0.95, balanced accuracy > 0.92 (five-fold cross-validation). - Purpose is actionable, enabling timely intervention before disengagement to improve retention. - Closest analog to Baby2Home's "disengagement/dropout prediction" candidate feature; models the timing of dropout, which is what a care-manager alert would need to surface.	
Van Mens K, Lokkerbol J, Wijnen B, Janssen R, de Lange R, Tiemens B. Predicting Undesired Treatment Outcomes With Machine Learning in Mental Health Care: Multisite Study. JMIR Med Inform. 2023 Aug 23;11:e44322. doi: 10.2196/44322. PMID: 37623374; PMCID: PMC10466445.	https://pmc.ncbi.nlm.nih.gov/articles/PMC10466445/	AI in Healthcare	Early non-response flagging through routine outcome monitoring	**Background**: Routine outcome monitoring can reveal patterns of early treatment response and help identify patients unlikely to benefit from their current treatment. Earlier, more accurate identification of these non-responders is needed so patients do not endure periods of ineffective care and scarce resources are not spent on treatments without the desired effect. **Methods**: This multisite study applied machine learning to routine outcome-monitoring data to predict undesired treatment outcomes (non-response) in mental health care. Drawing on data across multiple sites, the authors evaluated predictive performance for identifying patients at risk of poor outcomes and examined how such models could be integrated into measurement-based care, explicitly weighing the clinical costs of misclassification. **Results**: Machine learning applied to routine outcome data was able to flag patients at elevated risk of non-response, providing an early, data-driven signal for treatment adjustment. The study emphasized the trade-off inherent in these predictions: incorrectly classifying a patient as needing more intensive treatment consumes resources on patients who would have benefited from shorter, lower-intensity care, so model outputs must be balanced against misclassification costs. **Conclusions**: Machine learning on routine outcome-monitoring data can support earlier identification of likely non-responders and inform timely decisions about intensifying or changing treatment. Integrated into measurement-based care, such models can strengthen treatment-to-target workflows and caseload prioritization while remaining attentive to the costs of misclassification.	- Applies machine learning to routine outcome monitoring (ROM) data to predict non-response early in treatment. - Multisite design, strengthening generalizability across care settings. - Goal is timely treatment adjustment; intensify or switch before patients endure ineffective care. - Explicitly frames the misclassification trade-off: false positives waste resources on patients who would have improved with shorter, lower-intensity care. - Positions the models within measurement-based care to support treatment-to-target decisions. - Directly supports the "risk-based prioritization / flag patients not improving" dashboard feature. - Grounds the analytic logic behind the Systematic Caseload Review your registry dashboard is built to support.	

Raja, A. C. R. M. N. (2024). Proactive Care Management of AI-Identified At-Risk Patients Decreases Preventable Admissions. American Journal of Managed Care. https://link.gale.com/apps/doc/A815617892/AONE?u=umuser&sid=bookmark-AONE&xid=0615b160	https://go.gale.com/ps/i.do?id=GALE%7CA815617892&v=2.1&it=r&u=umuser&p=AONE&aty=ip	AI in Healthcare		Objectives: We assessed whether proactive care management for artificial intelligence (AI)–identified at-risk patients reduced preventable emergency department (ED) visits and hospital admissions (HAs). Study Design: Stepped-wedge cluster randomized design. Methods: Adults receiving primary care at 48 UCLA Health clinics and determined to be at risk based on a homegrown AI model were included. We employed a stepped-wedge cluster randomized design, assigning groups of clinics (pods) to 1 of 4 single-cohort waves during which the proactive care intervention was implemented. The primary end points were potentially preventable HAs and ED visits; secondary end points were all HAs and ED visits. Within each wave, we used an interrupted time series and segmented regression analysis to compare utilization trends. Results: In the pooled analysis of high-risk and highest-risk patients (n = 3007), potentially preventable HAs showed a statistically significant level drop (−27% [95% CI, −44% to −6%]), without any corresponding change in trends. Potentially preventable ED visits did not show a substantial level drop in response to the intervention, although a nonsignificant differential change in trend was observed, with visit rates decelerating 7% faster in the intervention cohorts (95% CI, −13% to 0%). Nonsignificant drops were observed for all HAs (−19% [95% CI, −35% to 1%]; P =.06) and ED visits (−15% [95% CI, −28% to 1%]; P =.06). Conclusions: A care management intervention targeting AI-identified at-risk patients was followed by a onetime, significant, sizable reduction in preventable HA rates. Further exploration is needed to assess the potential of integrating AI and care management in preventing acute hospital encounters.		
Tran, M. (2026). The CARE Model: A Framework for Artificial Intelligence Integration in Chronic Disease Management. Journal of Healthcare Management, 71 (4), 235-243. doi: 10.1097/JHM-D-26-00132.	https://oce.ovid.com/article/00115514-202607000-00004/HTML	AI in Healthcare				
Hwang M, Zheng Y, Cho Y, Jiang Y AI Applications for Chronic Condition Self-Management: Scoping Review J Med Internet Res 2025;27:e59632 URL: https://www.jmir.org/2025/1/e59632 DOI: 10.2196/59632		AI in Healthcare				